

One branch's week — the same week, after

The same branch, the same week, and what it is able to do instead.

- Intake

HCHB
- Insurance & Auth

Per Diem / Float
- PCC / Scheduler

Patient
- Clinician

Branch leadership
- DCS

TRIGGER

Monday, 7am
the queue as it stands

1 · THE QUEUE · MONDAY MORNING

Notifications fire on change only

- not on the same state each day
- the volume falls away
- what remains needs an answer

The scheduler works exceptions

- urgency, coverage, local knowledge
- the work only a person can do
- fewer people, better used

Welcome calls go out first thing

- standardised, and early
- before the day fills
- readiness known while it is useful

Care team assigned once, at referral

- not once per discipline
- the task burst never forms
- approval is a single step

THE QUEUE ONLY CARRIES DECISIONS

SCHEDULING, NOT ADMINISTRATION
The queue holds decisions, and the day holds judgment work.

2 · THE CALL-OUT · WEDNESDAY, 7AM

A clinician calls out at seven

- the same six visits
- the same two time-sensitive
- nothing about this changes

The branch can see who has room

- live, across the whole branch
- approval and discipline checked
- including flexible visits

Two covered from existing capacity

- clinicians already on the payroll
- who had room and were near
- offered, and they accepted

Premium used once, not four times

- for the visit that genuinely
- had no other option
- the rest cost nothing extra

COVERAGE IS FOUND BEFORE IT IS BOUGHT

PREMIUM SPEND, PLANNED
Bought where it was needed, not where it was quickest.

3 · A REFERRAL ARRIVES · THURSDAY

Referral offered by the hospital

- the same referral
- the same day
- the same people available

Admitting capacity shown honestly

- routine work that could move
- assistants with room to take it
- the true number, not the visible one

Routine visits shift to assistants

- only where they have capacity
- and the right qualification
- the nurse reviews and accepts

The referral is accepted

- an admission the branch would
- otherwise have declined
- no one was hired

ACCEPTED, USING THE SAME PEOPLE

AN ADMISSION GAINED
The capacity was always there. Now it can be seen and used.

4 · THE WEEK IN REVIEW · FRIDAY

The branch reviews the week

- the same accurate figures
- plus the ones that were missing
- capacity offered against filled

Premium spend shows its reason

- reactive or planned, per event
- a visibility problem is visible
- and therefore fixable

Declines are recorded with a reason

- capacity, clinical, or geography
- counted, and trended
- the argument for hiring gets evidence

The week can be improved

- because it can be measured
- the same week, next week,
- is a decision, not a hope

THE LOSSES BECOME NUMBERS

MEASURED, THEREFORE MANAGEABLE
The first week the branch could see what it lost.

WHAT CHANGED

1 THE QUEUE

- a queue that only carries
- things needing a decision

2 THE CALL-OUT

- premium bought once, where
- it was genuinely needed

3 THE REFERRAL

- accepted, by moving routine
- work to assistants with room

4 THE REVIEW

- losses counted, so the week
- can actually be improved

LEVERS ON THIS SHEET

- L1 Admission Throughput
- L2 Assessment Capacity Release
- L6 Scheduling Administration Cost
- L7 Premium Labor Avoidance